

Tentative Rulings
August 24, 2026
Department S-17
Judge Joseph Ortiz

Tentative Rulings for Department S-17 are posted on the court's website (<https://www.sb-court.org/divisions/civil/civil-tentative-rulings>) at 3:00 p.m. or at 7:00 p.m. the court day before the hearing. If no tentative ruling is posted at 3:00 p.m., please check again after 7:00 p.m.

Unless you wish to submit on the tentative ruling, you must appear for the hearing either in person, CourtCall (888-882-6878 or www.courtcall.com), or by ZOOM. Failure to appear is deemed a waiver of oral argument. If you wish to submit to the tentative, please call the Judicial Assistant at (909) 708-8715 in advance of the hearing. If all parties submit on a tentative ruling, it will become final. The tentative ruling may seek input on particular issues and direct appearance. If so directed, attendance at the hearing is mandatory. The party prevailing on a motion or other hearing shall serve written notice of the court's ruling unless all parties waive notice of the ruling.

ATTENTION: Since January 9, 2023, the court no longer provides an official Court Reporter to transcribe proceedings. Parties who wish to have a transcript must retain their own private reporter and must submit a "Stipulation and Order to Use Certified Shorthand Reporter." Please contact the Department if you need this form. Parties who do not retain their own reporter have waived the right to one.

UNLESS OTHERWISE NOTED, THE PREVAILING PARTY IS TO GIVE NOTICE OF THE RULING.

15. *Guy v. Loma Linda Univ. Med. Cntr., et al*, Case No. CIVSB2607407
Defendant Loma Linda's Demurrer
8/24/26, 9:00 a.m., Dept. S-17

This matter was continued from June 10, 2026, to allow meet-and-confer efforts

Tentative Ruling

The Court would **SUSTAIN** the demurrer as to Plaintiff's second cause of action for general negligence and third cause for medical battery. The Court would grant 30 days leave to amend.

Case Summary

This is a medical malpractice action. Plaintiff alleges that on or about January of 2025, he was hospitalized at Defendant's facility. During that surgery, he alleges he underwent emergency surgery and wound care. He asserts that Defendant failed to properly account for, count, or remove all surgical materials and that it negligently closed the wound or surgical site with gauze or foreign material remaining within Plaintiff's back. Plaintiff alleges that, following the procedure, the wound or surgical site did not heal or close properly, and he continued to experience pain, drainage, irritation, etc. He asserts that approximately five months later, on or about May of 2025, another medical practitioner identified and removed the gauze and other items left inside his back. As such, he filed suit on March 6, 2026, alleging (1) professional negligence; (2) negligence; (3) battery; and (4) negligent hiring and supervision.

Analysis

General Negligence (2nd Cause) – Defendant argues that the general negligence claim fails because it is wholly duplicative of the professional negligence claim and alleges no independent duty outside of rendering medical care. The Court agrees.

The opinion in *Williams v. Superior Court* (1994) 30 Cal.App.4th 318, is instructive: The *Williams* court concluded that "[t]o find whether an action arises out of the professional negligence of the health care provider, the 'allegations that identify the nature and cause of a plaintiff's injury must be examined to determine whether each is directly related to the manner in which professional services were provided.' [Citation.]" (*Williams, supra*, 30 Cal.App.4th, at p. 325.) The *Williams* court also held "that it is not the degree of skill required but whether the injuries arose out of the rendering of professional services that determines whether professional as opposed to ordinary negligence applies." (*Id.* at p. 327.)

Additionally, a party cannot evade the limitations of MICRA by asserting non-MICRA causes of action, as "the courts must determine whether it is nevertheless based on the 'professional negligence' of the health care provider so as to trigger MICRA." (*Unruh-Haxton v. Regents of University of California* (2008) 162 Cal.App.4th 343, 353.) Courts "must focus on the nature or gravamen of the claim, not the label or form of action the plaintiff selects." (*Larson v. UHS of Rancho Springs, Inc.* (2014) 230 Cal.App.4th 336, 347.) Causes of action that challenge the manner in which the professional rendered health care services, and that do not allege "some collateral course of conduct pursued for [the professional's] own gain or gratification," are subject to MICRA. (*Id.* at p. 352.)

Here, Plaintiff alleges that, in addition to professional negligence, Defendant owed Plaintiff “a duty to use reasonable care in the ownership, operation, maintenance, staffing, and control of [Defendant’s facilities], including in implementing and enforcing reasonable policies and procedures to prevent retained surgical foreign objects and to provide appropriate postoperative evaluation and wound care.” (Compl., ¶133.) Plaintiff further alleges that Defendant breached these duties and committed ordinary negligence by “failing to maintain and enforce adequate sponge and instrument count policies and procedures; failing to provide adequate numbers of properly trained staff in the operating room; failing to properly supervise those responsible for sponge and instrument counts; failing to maintain reasonable systems for postoperative follow-up and escalation of care when a wound fails to heal; and failing to promptly and adequately respond to Plaintiff’s repeated reports and presentations for a non-healing wound.” (Compl., ¶134.)

Although Plaintiff argues that his ordinary negligence claim is distinct from his claim for professional negligence because there were separate institutional failures stemming from the operation, maintenance, staffing, and control of Defendant’s facilities, these allegations – alone – do not transform such a claim to a claim for ordinary negligence. Such conduct is not collateral to rendition of professional services for the treatment of Plaintiff’s stab wounds. Thus, the Court would sustain as to this cause of action.

Battery (3rd Cause) – Medical battery occurs when a doctor performs a procedure or operation to which the patient has not consented or when consent is to one type of procedure or surgery and another type is performed. (*Cobbs v. Grant* (1972) 8 Cal.3d 229, 240-241.) “However, when the patient consents to certain treatment and the doctor performs that treatment but an undisclosed inherent complication with a low probability occurs, no intentional deviation from the consent given appears; rather, the doctor in obtaining consent may have failed to meet his due care duty to disclose pertinent information. In that situation the action should be pleaded in negligence.” (*Cobbs, supra*, 8 Cal.3d at pp. 240-241.)

To establish medical battery without consent, one must prove: (1) the defendant performed a medical procedure without the plaintiff’s consent or the plaintiff consented to one medical procedure but the defendant performed a substantially different medical procedure; (2) the plaintiff was harmed; and (3) the defendant’s conduct was a substantial factor in causing the plaintiff’s harm. (CACI, 530A.)

Here, Plaintiff alleges that Defendant committed medical battery in January of 2025 by leaving foreign objects inside his body during his stab wound care surgery. (Compl., ¶¶13-15, & 39-40.) Plaintiff alleges that, although he consented to the emergency surgical and wound care treatment, he did not consent to Defendant leaving any foreign object inside his body. (Compl., ¶¶13-14, 37-38, & 41.)

Defendant argues that Plaintiff fails to sufficiently state a cause of action for medical battery because Plaintiff admits that he consented to the surgical procedure performed by Defendant. Defendant points out that there are no allegations that Defendant performed a different operation, exceeded the scope of the procedure authorized by Plaintiff, or proceeded in the face of an express refusal of consent. Defendant argues that, although Plaintiff alleged that he “did not consent to the retention of a foreign object,” this allegation misstates the governing legal standard: It argues that the relevant inquiry is not whether Plaintiff consented to a particular outcome or complication, but whether he consented to the procedure performed.

In opposition, Plaintiff argues that the allegation that he did not consent to Defendant leaving any foreign object inside his body is sufficient to allege medical battery claim. Plaintiff argues that the determination as to whether a substantially different medical procedure was performed is a factual question unsuitable to be determined on demurrer, citing *Kaplan v. Mamelak* (2008) 162 Cal.App.4th 637, 647 [holding that whether a surgery is substantially different is a factual question not capable of being decided on demurrer] and *So v. Shin* (2013) 212 Cal.App.4th 652, 669 [holding that whether the contact fell within the scope of consent was a factual question for the finder of fact and not properly decided on demurrer]. However, Plaintiff's reliance on each of these cases is misplaced.

In *Kaplan*, the patient gave permission to the surgeon to operate only on a specific spinal disk (i.e., T8-9). (*Kaplan, supra*, 162 Cal.App.4th at p. 645.) The *Kaplan* court held that the surgeon may have committed battery by operating on the T6-7 and T7-8 disks when he did not have permission to operate on any disk other than T8-9. (*Id.* at p. 646.) Here, *Kaplan* is distinguishable because, here, Plaintiff did not specifically impose limitations as to the specific location within his body as to where the surgery was to be performed. Further, Plaintiff does not allege that Defendant performed the surgery at a different location on his body.

In *So*, the patient was admitted to a hospital following a miscarriage and the patient awoke during a procedure because she was administered insufficient anesthesia. (*So, supra*, 212 Cal.App.4th at p. 657.) The patient alleged that the anesthesiologist committed assault and battery during a postoperative interaction where the anesthesiologist attempted to persuade plaintiff not to report that plaintiff had awoken during surgery. (*Id.* at p. 670.) The patient alleged that the anesthesiologist's menacing conduct, of making movements to bring a container containing blood and other materials closer to Plaintiff's face, and her subsequent touching of Plaintiff's hands, arms, and shoulder were not consented to, either expressly or impliedly. (*Id.* at p. 671.) As such, *So* is distinguishable the *So* defendant's conduct occurred outside the scope of a medical procedure.

Here, by contrast, Plaintiff alleges no different procedure, no unauthorized surgery, and no treatment of a different body part. Plaintiff's allegations concern the manner in which the authorized medical procedure was performed, and not the performance of a substantially different medical procedure. Thus, the Court would sustain the demurrer as to this cause of action.

16. *Just N Time Trucking LLC v. Eaton Cummins Auto. Trans. Tech.*, Case No. CIVSB2503409
Defendant's Demurrer to Third Amended Complaint
8/24/26, 9:00 a.m., Dept. S-17

Tentative Ruling

The Court would **SUSTAIN**, without leave to amend.

Case Summary

This is a breach of warranty case. Plaintiff alleges that it purchased a 2019 commercial truck in October of 2018. Defendant sold the truck. Plaintiff further alleges that the sale included a warranty on the transmission for 60 months or 750,000 miles. However, Plaintiff asserts the truck was delivered with serious transmission defects. As such, it initially filed suit on February 3, 2025. The currently Second